



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : اسراء كمال كامل حسانين
المحافظة : المنيا
الحى / المدينة : ابوقرقاص
رقم البطاقة : 29608232400363 رقم التليفون : 01122437160
العمر (تاريخ الميلاد) : 23-08-1996 المهنة : حاصل علي بكالوريوس
العنوان : بني حسن الشروق
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و
بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: اسماء كمال كامل حسانين National ID: 29608232400363	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

ECHO REPORT

NAME: اسماء محمد DATE: 11/8/2026
ID NUMBER: C792/K-C7 SEX: ♀ AGE: 29Y

EF	70%	55-75%	PWT		0.6-1.1cm
LVEDD	4.2	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVEDS	2.1	2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

Scientific Airt. Outlet
e' mild. MR. (Rheumatic)

mild TR

RVF = 23 mmHg

Rheumatic

DR.

جمهورية مصر العربية
بطاقة تحقيق الشخصية

اسماء
كمال كامل حسنين
بنى حسن الشروق
مركز ابو قرقاص - المنيا

١٩٩٦/٠٨/٢٣ ٢٩٦٠٨٢٣٢٤٠٠٣٦٣

١٨٧٧٥٨٩٦٧

٢٠٢٠/٠١ ٢٩٦٠٨٢٣٢٤٠٠٣٦٣

بكالوريوس تربية رياضية

أنثى مسلمة متزوجة
محمود فوزى محمد خليفه

البطاقة سارية حتى ٢٠٢٧/٠١/٠٣ ٦

07/22/2026

MDT FORM

Name:	Kamel Kamel	HRN:	12530
Age:	30 Y	ID:	29608232400363
Menopausal Status:	premenopausal	Phone:	01122437160
NRS SCORE:		Marital Status:	married
BMI:		Family History:	Mother breast ca, M. cousin breast ca
Gender:		Reason Of MDT:	Newly diagnosed case
CO Morbidity:	+ve	ECOG Performance Status:	0 1 2 3 4
Surgical History:	C-section twice		

Clinical Data:

EX: IDC GIII
ER 7/8 PR 5/8 HER 3+ K16



History:

The condition started 6 months ago (during lactation), by accidental discovery of a Rt breast mass...pt sought medical advice with no improvement...ultrasoundography & biopsy revealed IDC ER7/8 PR6/8 Her2 3+...at Bahy el-Sayoum biopsy
Mammasonography (1/7/2025)....Rt breast LQO mals with level 1 & II pathological LNs + Lt breast
Biomarker/Molecular

Met's Workup:

CT CAP, Priminey Rt IMG LNs (largest 7.5mm)
K27 HFL 3mm...
MRI abdomen...hemangioma (flash filling)

Recommendations:

For: 1.Revision of pathology & IHC 2.CESM (pt weaned her child 1 month ago) 3.MDT to start

MDT Outcome & Plan

For:
1.Revision of pathology & IHC
2.CESM (pt weaned her child 1 month ago)
3.Portacath insertion
4.MDT to start NACT TCHP x6 (+Zoladex)
Re-MDT after CESM for surgical decision.

	Right Side				Left S	
	Q2	Q2	Q2	Q4	Q1	Q1
4x5cm						
12 o'clock						

Clinical Staging:

cT2 N3b M0

Pathological Staging:

NA

SIGNATURE

PATIENT NAME: ASRAA KAMAL KAMEL HASANEN

GENDER: Female

DATE : 02-Jul-2026 12:05 PM

PATIENT ID: 10126282460

AGE: 29 y

ORDER: 3937952,3937953

Multislice post contrast CT scan of the chest, abdomen and pelvis revealed:

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 50 ml.

FINDINGS:

(I) Chest:

- The right breast shows a deep enhancing mass lesion measuring about 36 x 27 mm in its maximum axial dimensions, indenting the underlying pectoral muscle with no evident infiltration.
- Ipsilateral enlarged right axillary lymph nodes are seen in level I and II, the largest is seen measuring about 15 x 15 mm in its maximum diameter.
- Prominent right internal mammary lymph nodes are seen, the largest measures about 7.5 mm in its maximum diameter.
- A tiny 3 mm pulmonary nodule is seen in the lateral segment of the left lower lung lobe, likely insignificant.
- Otherwise, Both lung fields are clear showing no parenchymal nodules, cavitory lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density and enhancement with:
 1. Arterial enhancing small lesion measuring 5 mm and not appreciated in the other phases.
 2. A geographic area of hypodensity is seen in the anterior aspect of

PATIENT: ASRAA KAMAL KAMEL HASANEN, 10126282460, Female, 29 y

Report version 2, 04-Jul-2026 01:31 PM

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مركز بهية الهرم
و شارع طلحة أول شارع الهرم بجوار داي . الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحى الأول - الشيخ زايد لنام كلية الهندسة

CT Scan
[[DOCTYPE:CT_SCAN]]

segment IV and V, with no evident contrast uptake, ?? focal fat infiltration

- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- IUD is noted
- Right adenexal cyst is seen , ?? functional.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Mild spondylodegenerative changes
- left 2nd rib sclerotic patch.
- No CT evidence of destructive osseous lesions, soft tissue components or vertebral collapse.

OPINION:

- Right breast enhancing mass lesion with Ipsilateral enlarged right axillary lymph nodes are seen in level I and II.
- Prominent right internal mammary lymph nodes.
- No CT evidence of definite pulmonary metastatic deposits.
- Arterial enhancing small left lobar hepatic focal lesion, ?? Flash filling hemangioma with geogragrahic hypodense area for better dynamic MRI assessment.
- Left 2nd rib sclerotic patch for bone scan correlation

Reported By,

Dr. Ahmed Abdel-Monem, MSc

Dr. Mennatallah Mohamed Hanafy, MD

04-Jul-2026 01:31 PM

PATIENT: ASRAA KAMAL KAMEL HASANEN, 10126282460, Female, 29 y

Report version 2, 04-Jul-2026 01:31 PM

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مركز بهية الهرم
4 شارع علوية اول شارع الهرم بجوار داري . الجيزة
مستشفى بهية الشيخ زايد
10 الحى الأول - الشيخ زايد امام كلية الهندسة



PATIENT NAME: ASRAA KAMAL KAMEL HASANEN

GENDER: Female

DATE : 13-Jul-2026 01:06 PM

PATIENT ID: 10126282460

AGE: 29 y

ORDER: 3951254

DYNAMIC MRI OF THE ABDOMEN

MRI TECHNIQUE:

- Axial T1, T2 WI and STIR.
- After I.V. injection of Gd-DTPA: dynamic axial T1 WI.
- Diffusion WIs and ADC map.

MRI FINDINGS:

- Markedly enlarged liver showing few bi-lobar hepatic focal lesions as follows:
 1. Segments VIII, V and III hepatic focal lesions eliciting low T1, high T2 and STIR signal with no diffusion restriction and early filling at the arterial phase.
 2. Segment III hepatic focal lesions eliciting low T1, high T2/STIR with diffusion restriction and peripheral nodular enhancement at the arterial phase with filling in the delayed phase.
- No dilated intrahepatic or extra-hepatic biliary radicles.
- Congested IVC with preserved signal void and enhancement pattern.
- The portal vein, hepatic veins and aorta all show signal void pattern denoting their patency.
- Normal MRI appearance of pancreas, adrenals, spleen and both kidneys.
- No sizable abdominal lymph nodal enlargement.
- No ascites.

OPINION:

- **Marked hepatomegaly with segment VIII, V, III and few hepatic focal lesions with enhancement pattern of Flash filling hemangiomas.**
- **segment III tiny hepatic focal lesion with enhancement pattern as described , likely benign in nature**

PATIENT: ASRAA KAMAL KAMEL HASANEN, 10126282460, Female, 29 y

Report version 2, 15-Jul-2026 06:05 PM

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مركز بهية الهرم
4 شارع طلحة أول شارع الهرم بجوار داري ، الجيزة

مستشفى بهية الشيخ زايد
1- الحي الأول - الشيخ زايد لأمم كلية الهندسة

PATIENT: ASRAA KAMAL KAMEL HASANEN, 10126282460, Female, 29 y

Report version 2, 15-Jul-2026 06:05 PM

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مركز بهية الهرم
4 شارع ملوية نول شارع الهرم بجوار داري , الجزيرة

مستشفى بهية الشيخ زايد
1- البني الأول - الشيخ زايد لتمام كلية الهندسة



PATIENT NAME: ASRAA KAMAL KAMEL HASANEN

GENDER: Female

DATE : 14-Jul-2026 10:46 AM

PATIENT ID: 10126282460

AGE: 29 y

ORDER: 3951255

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected alllover the surveyed skeleton.

Both kidneys are normally visualized.

Diffuse warm uptake opposite bilateral frontal and parietal bones likely hyperostosis frontoparietalis

Conclusion:

No definite scintigraphic evidence of metastatic osseous deposits.

Reported By,

Prof. Mai Amr, MD

15-Jul-2026 06:58 PM



PATIENT NAME: ASRAA KAMAL KAMEL HASANEN

GENDER: Female

DATE : 01-Jul-2026 04:09 PM

PATIENT ID: 10126282460

AGE: 29 y

ORDER: 3937509,3937842

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast mass of rapid increased size over 6 months. Strong positive family history. Patient recently stopped lactation in the right breast 6 months ago and left 10 days ago.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

Right breast:

- Heterogeneously dense breast c(ACR B).
- UOQ dense spiculated mass lesion with overlying amorphous microcalcifications seen about 3.9cm distance from nipple at CC view;
- Corresponds by US to 12:00 hypoechoic spiculated mass lesion with intense internal vascularity and few echogenic foci. It measures 4.2x2.8cm seen abutting the overlying skin , about 0.2cm from underlying chest wall muscles.
- Other small irregular hypoechoic lesion seen at 11:00 zone c measuring 0.9x0.5cm?? Metastatic IMLN.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Few level I & II globular shaped axillary lymph nodes; largest measures 1.6x1.3cm.
- No pathologically enlarged infra- or supraclavicular lymph nodes.

Left breast:

- Extremely dense breast (ACR d).
- No spiculated mass lesions or grouped microcalcifications.
- Lactational breast changes in the form of increased parenchymal echogenicity and dilated central ducts
- No solid or cystic mass lesions.
- Normal skin thickness and contour.

PATIENT: ASRAA KAMAL KAMEL HASANEN, 10126282460, Female, 29 y

Report version 1, 01-Jul-2026 05:01 PM

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مركز بهية الهرم
في شارع طلوع الشمس بجوار دارى العبد

مستشفى بهية الشامخ زنت
في الحي الاول - القصر - زنت امام كلية الهندسة

Mammogram

[[DOCTYPE:MAMMOGRAM]]

- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Right breast UOQ highly suspicious lesions associated with ipsilateral level I & II pathological lymph nodes (BIRADS 4); For histopathological correlation
- Left breast lactational breast changes with no suspicious lesions (BIRADS2).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Ghada Ali, MD

01-Jul-2026 05:01 PM



MRN:	10126282460	D.O.B:	23/08/1996
Patient Name:	ASRAA KAMAL KAMEL HASANEN	Sex:	Female
IC No.:	29608232400363		
Location:	External Clinic		

SURGICAL PATHOLOGY
Biopsy Small - (1 Site)

Specimen Id	2607020282	Collection Date	02/07/2026 03:44 PM
Accession No	SUR-26-2299	Received Date	02/07/2026 03:45 PM
Sample Type	Tissue	Doctor	radwa alaa eldien Mohamed abdelfattah

Clinical Information

Rt breast 12 o'clock highly suspicious lesion (BIRADS 5).

Gross Picture

Tissue cores 1-1.7 cm

Microscopic Picture

A case of invasive duct carcinoma, grade 3. No DCIS component nor lymphovascular emboli were detected. Stroma is desmoplastic with moderate inflammatory cell reaction(TILs 30%) with focal tumour necrosis.

Diagnostic Summary**Tumour type:**

Invasive duct carcinoma

Laterality

Right

Specimen type:

US-guided biopsy

Location

12 o'clock

Histological grade**Mitosis score (1-3):**

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Diagnosis

Rt breast (12 o'clock), US-guided biopsy :

Invasive duct carcinoma, grade 3.

Comments

ER, PR and HER2 is ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Marwa Mahmoud Abd Elkader, FEBP , IFCAP.

Co-signed by:

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

MRN:	10126282460	D.O.B:	23/08/1996
Patient Name:	ASRAA KAMAL KAMEL HASANEN	Sex:	Female
IC No.:	29608232400363		
Location:	External Clinic		

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2607050017	Collection Date	05/07/2026 08:57 AM
Accession No	IMM-26-2653	Received Date	05/07/2026 08:57 AM
Sample Type	Paraffin Block	Doctor	Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number

2299/26

Clinical Data

Rt. breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER: Positive, strong nuclear reaction in 35% of tumor cells [7/8].

PR: Positive, strong nuclear reaction in 25% of tumor cells [6/8].

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

MRN:	10126282460	D.O.B:	23/08/1996
Patient Name:	ASRAA KAMAL KAMEL HASANEN	Sex:	Female
IC No.:	29608232400363		
Location:	External Clinic		

IMMUNOPATHOLOGY
Her2 Receptors

Specimen Id	2607050018	Collection Date	05/07/2026 08:57 AM
Accession No	IMM-26-2654	Received Date	05/07/2026 08:57 AM
Sample Type	Paraffin Block	Doctor	Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
2299/26

Clinical Data
Rt. breast cancer

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:
Right

Laterality :
Right

Her2 Result
Positive [score 3+]

ECHO REPORT

NAME: اسماء محمد كامل DATE: 11/8/2026
ID NUMBER: C792/C-C7 SEX: ♀ AGE: 29Y

EF	<u>70%</u>	55-75%	PWT		0.6-1.1cm
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LVESD	<u>2.1</u>	2-4cm	LA	<u>2.5</u>	3-3.7cm
FS	<u>0</u>	25-45%	RV	<u>3.0</u>	2-4cm
SWT		0.6-1.1cm	AO.CUSP		0.7-2.3cm(supine)
					1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

Scientific Ant. defect
1 mild MR (Rheumatic)
mid TR
RVF = 23 mmHg

Rheumatic

DR.